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IN THE SUPREME COURT OF IOWA

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SUPREME COURT NO: 25-1433

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MICHAEL WOOFF

Petitioner-Appellant,

vs.

NINE PARK LANE, INC. d/b/a MENEFEE DRYWALL and WESTERN NATIONAL INSURANCE COMPANY
Respondents-Appellees

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APPEAL FROM THE IOWA DISTRICT COURT
IN AND FOR LINN COUNTY
Case No: CVCV106242
THE HONORABLE CHAD KEPROS

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# APPELLANT'S BRIEF

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**B. WHETHER SUBSTANTIAL EVIDENCE SUPPORTS THAT WOOFF'S MENTAL HEALTH INJURY SHOULD BE CONSIDERED IN THE INDUSTRIAL DISABILITY ANALYSIS**

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## STATEMENT OF THE ISSUE PRESENTED FOR REVIEW

**A. WHETHER THE COMMISSIONER COMMITTED AN ERROR OF LAW IN CONCLUDING THAT DEFENDANTS OFFERING WOOFF JOBS THAT HE WAS UNQUALIFIED TO DO WAS SUFFICIENT TO AVOID AN INDUSTRIAL DISABILITY AWARD**

**B. WHETHER SUBSTANTIAL EVIDENCE SUPPORTS THAT WOOFF'S MENTAL HEALTH INJURY SHOULD BE CONSIDERED IN THE INDUSTRIAL DISABILITY ANALYSIS**

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## ROUTING STATEMENT

This case involves questions relating substantial issues of first impression arising from the legislative changes made in 2017 to the laws governing Iowa Workers' Compensation claims. As such, the Supreme Court should retain this matter pursuant to Iowa Rule of Appellate Procedure 6.1101(2)(c).

## NATURE OF THE CASE
## STATEMENT OF THE CASE

Michael Wooff ("Wooff") filed a Petition for Arbitration seeking benefits for injuries to his right shoulder, cervical spine, and mental health. Defendants stipulated to Wooff's injury to the right shoulder (Hearing Report). Wooff's injuries to the cervical spine and mental health were disputed. The matter came for hearing on October 4, 2023. Deputy Andrew Phillips issued an Arbitration Decision on March 21, 2024, concluding that Wooff had met his burden of proving the cervical spine and mental health injuries were both work-related (03/21/24 Arb. Dec.). As a result, Wooff's injury was compensated on the basis of industrial disability and Deputy Phillips found Wooff sustained a 55% industrial disability (03/21/24 Arb. Dec., p. 60).

On Appeal, the Commissioner affirmed the finding that Wooff sustained right shoulder, neck, and mental health injuries as a result of his work activities but reduced the award to FUNCTIONAL IMPAIRMENT (rather than industrial disability) and awarded 41% (10/18/24, App. Dec., pp. 5-6). The Commissioner concluded that the Defendant employer offered Wooff employment "consistent with his restrictions" because he was "physically capable of performing" the job and "for which claimant would have received greater wages or earnings than he earned at the time of the work injury" (10/18/24, App. Dec., p. 4). Pursuant to Iowa Code Section 85.34(2)(v), the Commissioner concluded this offer takes the analysis into the "functional impairment" and out of "industrial disability" (10/18/24, App. Dec., p. 4). Wooff appealed to the District Court. The District Court held:

> *While Petitioner has argued that he did not have the mental abilities to perform the type of work offered to him by Respondents, Respondents could have, as Commissioner Cortese found, provided training to Petitioner to perform this work, and it does not appear Petitioner even tried the new jobs to see if he could be trained into performing them. The Court agrees with Commissioner Cortese that the statute does not leave it to a claimant to determine whether he or he is suitable for the proposed position, nor does it preclude an employer from providing training to an employee to attempt to get them into a new area of employment that could be suitable for the employee. The Court finds merit in Commissioner Cortese's conclusion that there is good policy that would encourage an employer to provide training in an effort to get an injured employee into a different and perhaps higher paying job so that the employment relationship can continue. The Court finds nothing in the record to suggest Respondents made the offer of employment in bad faith or an attempt to exclude Petitioner from*

> *employment, as opposed to making him an offer of something the employer viewed him to be suitable for and then providing training to adjust Petitioner to the new position(s). The employer was in a position to assess Petitioner's abilities with regard to what jobs he might be capable of performing, and the Court agrees with Commissioner Cortese that the offer was made in good faith. Petitioner's argument on this issue fails.*

(D0023, 07/14/25 Ruling on Judicial Review, pp. 11-12).

The Commissioner also did not consider impairment due to Wooff's mental injury health stating "Dr. Woods did not assign any permanent restrictions for the mental health injury" and that "claimant failed to prove specific permanent functional impairment under the AMA Guides to the Evaluation of Permanent Impairment, Fifth Edition, related to the mental health injury" (10/18/24, App. Dec., pp. 3, 5). Finally, if Wooff is limited to an award of functional impairment, the Commissioner erred as a matter of law by using agency expertise in determining Wooff's permanent impairment.

## STATEMENT OF THE ISSUES

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### STATEMENT OF THE FACTS

Michael Wooff ("Wooff") was 56 years old at the time of his hearing (Tr., p. 19). He is married to his wife, Kayla (Tr., p. 20). Kayla was employed as a 911 dispatcher for Johnson County (Tr., p. 73). Wooff is a high school graduate (Tr., p. 20). He served in the United States Marine Corps from 1987 to 1991 and was honorably discharged (Tr., p. 21). Wooff is right-handed (Tr., p. 22). Wooff worked with Menefee Drywall for almost 26 years, starting in 1993 until 2021 (Tr., p. 22). Wooff's job was a drywall finisher which he described as "after the drywall is hung, you go in and tape, mud, sand and get it ready for paint" (Tr., p. 23). He described this as "a very physical job" (Tr., p. 24). He had to be on stilts, lifted products, worked in physical demanding positions, and constantly moved his head, arms, and shoulders (Tr., pp. 24-25; Cl Ex. 1, pp 17, 34). His injury involves a stipulated cumulative injury that initially began with right shoulder pain that got to the point where he "couldn't handle the pain anymore" (Tr., p. 26). The injury has resulted in three (3) surgeries to his right shoulder, two (2) surgeries to his cervical spine, and depression (Jt. Ex. 5, pp 127-132; Jt. Ex. 7, pp. 251-254; 293-294; 399-402; 512-551).

Defendants claim Wooff was offered two jobs in August of 2021, one as an estimator and one as a superintendent. Wooff did not feel qualified for either position given that he had always only performed manual labor in drywalling (Tr., pp. 41-45; 63-64). Woof testified:

*Q. And what can you tell me about the estimator's job?*
*A. A lot of computer work, going over prints, estimating the costs and how much materials would be provided for that job.*
*Q. And did you think you were capable of doing that job?*
*A. No.*
*Q. Why not?*
*A. I don't have any computer --*
*Q. Training?*
*A. Training. There's the word, yes, training.*

(Tr., pp. 41-42).

*Q. What was the other position that you were offered?*
*A. A commercial supervisor.*
*Q. And what does that mean?*
*A. You are in charge of manning the jobs and then checking on the jobs, making sure you're staying up-to-date and on schedule, plus answering questions from the contractors as well as the employees.*

(Tr., pp. 42-43).

*Q. And did you feel capable of performing that job?*
*A. No, I did not.*
*Q. And why not?*
*A. Just the physical and mental demand of that job. At that point in time I knew I couldn't do it, plus I just didn't feel comfortable doing that job.*
*Q. Well, earlier you described kind of the two components as far as workers for Menefee. What, framers --*
*A. Correct.*
*Q. -- or carpenters and then the drywallers?*
*A. Uh-huh.*

*Q. You know, if you were going to supervise, you would be familiar with drywallers; correct?*
*A. Correct.*
*Q. And you would know whether somebody was getting the job done and whether it was done correctly?*
*A. Correct.*
*Q. But not the carpenters. Is that a job you were familiar with?*
*A. No.*
*Q. Is that a job you feel like you could have supervised?*
*A. No.*
*Q. Why not?*
*A. I don't have any experience with doing any framing or carpenter work.*
*Q. Would you know how to recognize whether things are getting done appropriately?*
*A. No.*

(Tr., pp. 43-44).

At that time, Wooff was still treating the DENIED NECK INJURY (which was found to be compensable). He was not at maximum medical improvement for all of his work injuries at the time of the purported job offers made in August of 2021. He had his first cervical surgery 4 months later, on October 18, 2021 (Jt Med. Ex. 7, pp. 409, 429). He was unable to work from October 18, 2021, to March 2022, (5 months). Since he could not go back and perform his job as a drywall finisher, he secured a job with Methwick Manor (Tr., p. 40). The job was full-time and consisted of light maintenance (Tr., p. 40). He found the job through friends and he lasted about six months in this job (Tr., p. 40). He testified the job did not go well for him, he would experience increased pain and would miss work or be unable to do the job (Tr., pp. 40-41).

Wooff still has not been able to return to gainful employment following his second cervical surgery on February 24, 2023 (Jt. Med Ex. 7, pp. 530-531; Cl. Ex. 4, p. 156). Defendants did not offer Wooff any work after he reached MMI for his cervical injury.

## EVIDENCE OF PERMANENT IMPAIRMENT FOR WOOFF'S PHYSICAL INJURIES

Authorized treating physician, Dr. James Nepola, found Wooff to have a 16% impairment to his right upper extremity which converts to 10% whole body impairment. The 16% included 10% due to the distal clavicle excision (Jt. Med Ex 7, p. 387).

Dr. David Segal found Wooff to have 35% impairment to his body as a whole due to his cervical injury and 13% body as a whole due to his right shoulder injury. Dr. Segal also rated the distal clavicle excision at 10% upper extremity (Cl. Ex. 1, pp. 67, 58, 60, 77).

Dr. Joseph Chen found Wooff to have 8.5-9% impairment to his upper extremity and did not rate the cervical injury. Dr. Chen rated the distal clavicle excision at 2.5% (Defs Ex. C, p. 37).

Dr. Adam Woods found Wooff to have a 40% impairment to his body as a whole due to his mental injury (Cl. Ex., 2, p. 140).

## EVIDENCE OF PERMANENT IMPAIRMENT FOR THE MENTAL HEALTH CONDITION

Wooff was evaluated by Dr. Adam Woods on August 8, 2023. Dr. Woods is a psychiatrist (Cl. Ex. 2, pp. 149-150). His comprehensive report outlines Wooff's relevant background history, discussing Wooff's work injury, and previous encounters with mental health (Cl. Ex. 2, pp. 133-141).

Dr. Woods diagnosed Wooff with a primary psychiatric diagnosis of Adjustment Disorder with Mixed Anxiety and Depressed Mood (Cl. Ex. 2, p. 136). Dr. Woods noted:

> *Mr. Wooff currently reports dramatically increased mental health symptoms on a daily basis, such as "feeling worthless" and having "nightly insomnia due to all the pain", which only exhausts him further and leads to more symptoms. Although he used to have some mild "social anxiety", Mr. Wooff now reports having overt panic attacks "many times per week" such that he has become scared to leave his home and be around other people* (Cl. Ex. 2, p. 138).
>
> Regarding employment, Dr. Woods opined:
>
> *Mr. Wooff's Adjustment Disorder presents him with significant hindrances in all the domains mentioned in this question. Especially in light of his chronic pian, weakness and mobility issues, the client's employability is greatly reduced by his labile moods, reduced concentration and energy, and his overall rapid exhaustion from dealing with the mental and physical anguish would make it very difficult for him to attend work with any reliability* (Cl. Ex. 2, p. 139).

7. *What effects, if any, would your diagnoses have on Mr. Wooff 's employability (mood, concentration, energy, ability to work with others, memory, ability to attend work on a regular basis)?*

*Mr. Wooff's Adjustment Disorder presents him with significant hindrances in all the domains mentioned in this question. Especially in light of his chronic pain, weakness, and mobility issues, the client's employability is greatly reduced by his labile moods, reduced concentration and energy, and his overall rapid exhaustion from dealing with the mental and physical anguish would make it very difficult for him to attend work with any reliability.* (Cl. Ex. 2, p. 139).

8. *In the event that Mr. Wooff continues to experience chronic pain from his August 9, 2019 work injury, without further relief of the chronic pain, is it more likely than not the condition(s), if any, you have diagnosed is permanent?*

   *Yes. Mr. Wooff's chronic pain is a big part of his ongoing Adjustment Disorder and its other related symptoms. If the pain is not resolved (or reduced to at least to a manageable level) then it is very likely (within a reasonable degree of medical certainty) that the Adjustment Disorder will be permanent.* (Cl. Ex. 2, pp. 139-140).

9. *Can the extent of loss or percentage or permanent impairment regarding Mr. Wooff 's mental health condition be determined by use of the AMA Guides?*

   *Yes. When considering the AMA Guides with respect to mental health impairment, it is perhaps most helpful to consider Table 14-1, which attempts to simplify the following paradigm. As mentioned in section 14.3, "there are no precise measures of impairment in mental disorders", however "there are some individuals with less than chronic, but still unremitting, impairments who are severely limited in certain areas of function". The AMA Guides then go on to outline "four main categories" to "assess…areas of function". These are (1) the ability to perform activities of daily living, (2) social functioning, (3) concentration, persistence, and pace, and (4) deterioration or decompensation in work or work-like settings. Table 14-1, therefore, presents five potential "classes" into which a person with a work-related injury may fall which incorporates these four elements and attempts to objectively quantify something which is admittedly subjective in nature.*

*In Mr. Wooff's case, it is my contention within a reasonable degree of medical certainty that he would fall into Class 3 or "Moderate Impairment". The AMA Guides define this category as "impairment levels [which] are compatible with some, but not all, useful functioning" and goes on to describe someone in the "moderate" class as someone who would be "limited in the ability to carry out many, but not all, complex tasks". When considering Mr. Wooff's Adjustment Disorder, especially in light of his mental health symptoms (and ongoing pain), this would seem to be the most precise category.*

*In terms of a "functional impairment rating", I would assign Mr. Wooff a 40% functional loss as a result of the work-related injury. This is due to his Class 3 "Moderate Impairment" and is modeled off the decision of "Clark v. Arconic, Inc." where Mr. Joseph Cortese, the Worker's Compensation Commissioner, found for a 30% functional loss for an individual deemed to have mild impairment. In addition, Table 11-4 from the AMA Guides was used as a model to help arrive at this rating. This is because (A) the AMA Guides do not provide rating percentages for mental health and (B) in Table 11-4 (like in Table 14-1) the Class 1 impairment is "no impairment". This MD therefore thought it prudent to use Table 11-4 as a model to provide at least some guidance for providing a numeric rating.* (Cl. Ex. 2, p. 140).

10. *Please address what, if any, permanent restrictions or limitations you would impose on Mr. Wooff due to his mental health conditions diagnosed in Question*

*To begin, with regard to my answer for Question #9, let me first state that the impairment rating given reflects how Mr. Wooff is doing at this moment in time. It is not necessarily permanent (and this MD is not able to assert that the client is at maximum medical improvement (MMI)), because his mental health treatment up to this point has not been optimal. It is probable he could make marked improvements if this were corrected and I recommend his care be transferred to Dr. Jeff Alden at the VA Outpatient Clinic in Coralville as soon as possible. Dr. Alden, a Psychiatrist with many years of general medicine experience as well, will not only treat Mr. Wooff's mental health ailments, but will serve as*

*a care coordinator and assist the client in getting the other services he needs (i.e., psychotherapy, pain treatment & therapy, etc.) to improve his overall health. Once the client has been receiving comprehensive mental health treatment for at least six months (possibly a year or more), only then can he truly be assessed for the possibility of MMI regarding his mental health ailments. For the sake of clarity, I will explain further below.*

*In mental health, determining a date for maximum medical improvement (MMI) is extremely challenging due to its multifactorial nature. To put it simply, it is always possible one's mental health symptoms could improve with a new medication regimen or substantial lifestyle changes, but the question of how likely and how much change is problematic to predict. With that said, in this case, Mr. Wooff's chronic pain, increased weakness, and mobility issues are a key part of his Adjustment Disorder and these facets seem extremely unlikely to fully resolve. Given this fact, it is therefore improbable his mental health symptoms will fully resolve either, but with the caveat that, as said earlier, that it is always possible, given the complexity of the brain and the numerous psychosocial factors at play, that Mr. Wooff's mental health symptoms may ebb and flow over time. Therefore, if a specific date is needed for my impairment rating given in Question #9, it would be August 8, 2023, the date of this MD's examination of Mr. Wooff. If the client does not get further treatment for his mental health ailments, then it would be fair to call that his date of MMI.*

*With all that said, this MD is highly doubtful that Mr. Wooff will be able to work a physical labor job ever again. Given his age and education level, it also seems improbable that he will be able to retrain for a non-physical, primarily cerebral line of work either. For the sake of his quality of life and peace of mind, we must proceed forward under the assumption he is not able to work at all at this point in time. This MD is not comfortable, however, assigning permanent restrictions/ limitations and will defer to Dr. Alden, the VA mental health staff, and my physical medicine colleagues to make such decisions.* (Cl. Ex. 2, pp. 140-141).

Dr. Segal addressed the mental health issue as well in his June 23, 2023, evaluation, specifically referring to the Commissioner's decision in Clark v. Arconic, Inc. and Indemnity Insurance Company of North America, File No. 5061553.01 (06/28/22 App. Dec.)(Cl. Ex. 1, p. 76). He diagnosed the mental health condition as the development of a permanent aggravation of his preexisting depression (Cl. Ex. 1, p. 70). He stated Wooff's symptoms included:

- *increased irritability, frustration, and restlessness as a result of this pain.*
- *moderate irritability/social withdrawal/low frustration tolerance.*
- *low energy/apathy insomnia with resulting fatigue*
- *moderate sadness/depression/anhedonia/lack of pleasure*
- *anxiety/agitation or nervousness*
- *His mood is depressed and anxious* (Cl. Ex. 1, p. 70).

Dr. Segal opined:

> *Yes, the work injury of August 9, 2019, and resulting chronic pain does constitute a substantial contributing factor in bringing about the permanent aggravation of Mr. Wooff's mental health symptoms* (Cl. Ex. 1, p. 73).

Dr. Segal further identified the effects on employability (Cl. Ex. 1, p. 74). He established an MMI date of June 24, 2023 for the mental health injury (Cl. Ex. 1, p. 75). He concluded:

> *In a recent case before the worker's comp commissioner, (Clark v. Arconic, Inc. and Indemnity Insurance Company of North America, File No. 5061553.01 (June 28, 2022) the Class of impairment found on Table 16-1 on page 363 was used as a basis for impairment rating. The commissioner assigned 30% impairment for Class 2 mild impairment. Mr. Wooff has mild impairment in ADLs, social functioning,*

> *concentration, and adaptation. Because of this, extrapolating from the commissioner's analysis, the 18% WPI for mental health function would be appropriate using this method as well* (Cl. Ex. 1, p. 76)(Citation added).

Defendants served a report on August 31, 2023, from a psychiatrist, Dr. Carpenter (Def. Ex. D). Critically important in Dr. Carpenter's statement:

> *...I have not examined the patient in person* (Def. Ex. D, p. 65).

Dr. Carpenter summarily dismisses any mental health issue being associated with the work injury, concluding Wooff's problems were preexisting (Def. Ex. D, p. 65). Dr. Carpenter fails to acknowledge the chronic effects of the work injury, including (5) surgeries that have dramatically altered Wooff's life and ability to work.

Regarding Dr. Carpenter's conclusions, Deputy Phillips held:

> *The defendants had a records review conducted by Dr. Carpenter. Without examining the claimant, Dr. Carpenter opined that the claimant had pre-existing depression and long-standing alcoholism. These resulted in no psychological or mental health injury caused by the August 9, 2019, work injury. I give Dr. Carpenter's opinion very little weight. He did not examine or even interview the claimant. The greater weight of the evidence indicates the opposite of the findings of Dr. Carpenter.*
>
> *At the time of the hearing, Mr. Wooff testified that his professed inability to work and provide for his family has worn on him. He also did not feel like undertaking any activities. He opined that he no longer enjoyed life. His wife testified, credibly, that Mr. Wooff is restless at night, and that he experienced "a lot" of crying spells. She also reported that Mr. Wooff conveyed a sense of uselessness since his work injury. She concluded*

*that Mr. Wooff's memory and concentration worsened since his work injuries.*

*The evidence indicates that Mr. Wooff had an issue with drinking. He addressed this issue, along with his depression in September of 2019. There is no indication that he has drank any alcohol since his hospitalization in September of 2019. For a time, his depression was stable. It was not until he was substantially into his medical treatment for his work-related injuries that his depression worsened. Dr. Woods' explanation is the most credible as to this issue. Dr. Woods actually examined the claimant and reviewed all of the pertinent records. Dr. Carpenter's report is rejected, as he never even interviewed the claimant. His only context is what was contained in the claimant's medical records. For something like a psychiatric claim, it would seem crucial for the provider to at least conduct an assessment of the claimant.*

*Based upon the evidence, I find that the claimant proved that his mental health issues were aggravated or lit up as a sequela of his right shoulder and neck injuries. Therefore, I find that his mental health issue arose out of, and in the course of his employment with the defendant. I also find that the claimant has shown this to be a cause of permanent disability based upon the foregoing reasons.*

(03/21/24 Arb. Dec., pp. 51-52).

## ARGUMENT

### STANDARD OF REVIEW

The Iowa Supreme Court has discussed the standard of review for different appellate grounds in the workers compensation system as follows:

> *If the claim of error lies with the agency's findings of fact, the proper question on review is whether substantial evidence supports those findings of fact. If…the claim of error lies with the ultimate conclusion reached, then the challenge is to the agency's application of the law to the facts, and the question on review is whether the agency abused its discretion by, for example, employing*

*wholly irrational reasoning or ignoring important and relevant evidence. See, Iowa Code § 17A.19(10)(i), (j). Meyer v. IBP, Inc., 710 N.W.2d 213, 218 (Iowa 2006) (emphasis added).*

The standard of judicial review of an agency decision is governed by Iowa Code chapter 17A, Iowa's administrative procedure act. See Iowa Code § 86.26; Quaker Oats Co. v. Ciha, 552 N.W.2d 143, 149 (Iowa 1996). The court's statutory scope of review provides for reversal if the Agency's decision was "unreasonable, arbitrary, capricious, or an abuse of discretion." Iowa Code § 17A.19(10)(n). An agency's action is arbitrary or capricious when "it is taken without regard to the law or facts of the case" and unreasonable when "it is clearly against reason and evidence." Soo Line R.R. v. Iowa Dep't of Transp., 521 N.W.2d 685, 688-89 (Iowa 1994). Unreasonableness is defined as action in the face of evidence that leaves "no room for difference of opinion among reasonable minds, or not based on substantial evidence." Stephenson v. Furnas Elec. Co., 522 N.W.2d 828, 831 (Iowa 1994). "Abuse of discretion is synonymous with unreasonableness, and involves lack of rationality, focusing on whether the agency has made a decision clearly against reason and evidence" Id.

The Courts are bound by the Agency's findings of fact if supported by substantial evidence. See Mosher v. Dep't of Inspections & Appeals, 671 N.W.2d 501, 508 (Iowa 2003). The Court may reverse, modify, or grant other relief when

agency action is based on fact determinations "not supported by substantial evidence in the record before the court when that record is viewed as a whole." Iowa Code § 17A.19(10)(f) (2001). "Substantial evidence" is statutorily defined as the quantity and quality of evidence that would be deemed sufficient by a neutral, detached, and reasonable person, to establish the fact at issue when the consequences resulting from the establishment of that fact are understood to be serious and of great importance. Id. § 17A.19(10)(f)(1). "On appeal, our task 'is not to determine whether the evidence supports a different finding; rather, our task is to determine whether substantial evidence ... supports the findings actually made." Cedar Rapids Community School District v. Pease, 807 N.W.2d 839, 845 (Iowa 2011).

### A. THE COMMISSIONER COMMITTED AN ERROR OF LAW IN CONCLUDING THAT DEFENDANTS' OFFERING WOOFF A JOB THAT HE WAS UNQUALIFIED TO DO WAS SUFFICIENT TO AVOID AN INDUSTRIAL DISABILITY AWARD

The Commissioner stated:

> *This case clearly involves a neck injury and a mental health injury. Neither of those conditions is accounted for, or is considered to be, a "scheduled member" injury pursuant to Iowa Code section 85.34(2)(a)-(u). Therefore, claimant has proven he sustained an "unscheduled" injury and that he should be compensated pursuant to Iowa Code section 85.34(2)(v).*
>
> *Iowa Code section 85.34(2)(v) provides, in pertinent part:*
>
> > *If an employee who is eligible for compensation under this paragraph returns to work or is offered work for which the*

> *employee receives or would receive the same or greater salary, wages, or earnings than the employee received at the time of the injury, the employee shall be compensated based only upon the employee's functional impairment resulting from the injury, and not in relation to the employee's earning capacity.*

(10/18/24 App. Dec., p. 4).

At the time of the hearing, Wooff was 56 years old with a high school diploma (Tr. p. 20). At the time of the 2019 injury, Wooff had worked as a drywall finisher for almost 26 years, starting at Menefee in 1993. He taped, mudded, sanded, and prepared already hung drywall for painting (Tr., p. 23). He had to be on stilts, lifted products, worked in physical demanding positions, and constantly moved his shoulders (Tr., pp. 24-25).

Defendants claim Wooff was offered two jobs in August of 2021, one as an estimator and one as a superintendent. Wooff did not feel qualified for either position given that he had always only performed manual labor in drywalling. After almost 26 years of experience with the job and Menefee as his employer, and after five major surgeries, his decision not to accept the job offers is not a refusal of suitable work. The job offers were not bona fide offers, given that Wooff was not qualified to perform them and was still in a healing period for his injuries.

The Commissioner held:

> *In this case, Menefee offered claimant two positions of employment after his injury. One of those employment offers was specifically put*

*into writing and is well-documented. Claimant appears physically capable of performing the supervisory position that the employer offered.*

*Under the terms of the offer made by Menefee, claimant would be paid a $60,000 annual salary to be a supervisor. This is more than he was earning at the time of this work injury. As such, I believe the employer demonstrated that an offer of employment was made to claimant for which claimant would have received greater wages or earnings than he earned at the time of the work injury. Claimant declined the employment offer from Menefee. Pursuant to the plain language of Iowa Code section 85.34(2)(v), claimant's permanent disability recovery in this case is limited to the "functional impairment resulting from the injury" and he should not be compensated with industrial disability based on a loss of earning capacity. I find defendants have demonstrated error in the arbitration decision and the decision must be modified.*

*Iowa Code section 85.34(x) states:*

*x. In all cases of permanent partial disability described in paragraphs "a" through "u", or paragraph 'v' when determining functional disability and not loss of earning capacity, the extent of loss or percentage of permanent* ***impairment shall be determined*** *solely* ***by utilizing the guides to the*** *evaluation of permanent impairment, published by the American medical association, as adopted by the workers' compensation commissioner by rule pursuant to chapter 17A. Lay testimony or agency expertise shall not be utilized in determining loss or percentage of permanent impairment pursuant to paragraphs "a" through "u", or paragraph "v" when determining functional disability and not loss of earning capacity.*

(10/18/24 App. Dec., p. 5)(emphasis added).

On Rehearing, the Commissioner stated:

*However, I address claimant's argument that a person can be "physically capable" and still not be "qualified." In the appeal decision,*

*I found that the employer must have considered claimant to be qualified. I also found that the job offer was a good faith offer. Claimant may have believed himself to be "unqualified," but the employer apparently believed claimant capable of performing the supervisory position. A finding of fact was made on the issue and no further findings are necessary.*

*Moreover, claimant's argument appears to read language into the statute that does not exist in a plain reading of the statute. Nothing in the statute would preclude an employer from making a job offer that would require additional training to assist an injured employee. Even if claimant may have felt unqualified, nothing in the statute precludes an employer from making a good faith offer of further employment that may require claimant to retrain or learn some new skills.*

*Nothing in Iowa Code section 85.34(2)(v) gives the claimant a unilateral option to determine his or her own "qualification" for a job offer. Claimant's argument would, in essence, give claimant veto power over a job offer by simply proclaiming his belief that he is "unqualified" for the job offered. Rather, sound policy would encourage an employer to try to retain its employees, perhaps even giving them a promotion and higher paying position as a supervisor to continue their employment. I reject claimant's reading of the statutory language to include language that is not in the plain text. The employer made a good faith job offer. Claimant rejected the job offer, and his recovery is now limited to a functional disability award. Iowa Code section 85.34(2)(v).*

*Claimant further challenges the applicability of Iowa Code section 85.34(2)(v) based on the timing of the employer's job offer. Claimant asserts that the job offer occurred prior to him achieving maximum medical improvement. Claimant contends in the motion for rehearing that Iowa Code section 85.34(2)(v) does not apply because "Defendants did not make another offer of work after Claimant reached MMI for [sic] after his cervical surgery."*

*Once again, claimant reads something into the statute that does not exist in the plain language of the text. Nothing in Iowa Code section 85.34(2)(v) limits or specifies when an offer of work can or must be*

> *made by an employer. Nothing in the plain language of the statute requires the job offer only be made after maximum medical improvement or that "another offer of work" be made after maximum medical improvement.*

(11/12/24 Ruling on Cl's Motion for Rehearing of App. Dec., p. 2).

Wooff was not educated, experienced, or mentally capable enough to do the jobs offered to him. Nor was he physically capable of working at the time of the offer. Simply because Menefee offered Wooff two jobs for which he was not qualified should not detract from his claim for industrial disability. While § 85.34(2)(v) does not use the terms "bona fide offer" or "suitable work," certainly such is implied. An employer cannot simply offer an employee ANY job to avoid the industrial disability analysis. The undersigned could physically sit in an airport control tower but does not have the training, experience, or qualifications to be an air traffic controller. The Deputy's "speculation" that "it appeared that Wooff was qualified for at least one of the positions based upon the job description, Mr. Wooff's experience, and his restrictions" and that "Mr. Wooff appears to be capable of retraining based upon his past experience" is simply that – speculation (03/21/2024 Arb. Dec., p. 56).

There is NO EVIDENCE IN THE RECORD that the employer was going to train Wooff for the job. Defendants' Exhibit E page 68 contains the job offer which simply states: "After reviewing your extensive work experience we would like to extend an offer as our new Superintendent. Your start date is September 1, 2021.

Attached is a copy of the job description, which is within your permanent work restrictions." (Cl. Ex. 7, pp. 294-297). Nor does Defendants' counsel's letter to the undersigned (at Defendants' Exhibit E page 69) discuss offering training or any reassurance that they will work with him to learn the jobs allegedly offered. All Wooff knew was that he was being offered a job that he knew he had no experience doing and was MENTALLY unqualified for.

It is the Commissioner who is reading things into the RECORD and the statute that are not there – that the job offer does not have to be currently suitable or that the job offer can require further training. Wooff's experience and 26-year career is in the manual labor portion of the drywall industry – not in employee management, hiring, firing, staffing, estimating, computer work, or knowledge suitable to supervise carpenter work of over 100 employees (Cl. Ex. 7, pp. 295-296). As a result of his injuries, Wooff is unable to perform any work within his experience, training, intelligence, and physical capacities. The Commissioner appears to be critical of Wooff, suggesting he is attempting to read into the statute meanings that do not exist in the plain language of the text. The plain language of Iowa Code § 85.34(2)(v) states:

> *If an employee who is eligible for compensation under this paragraph returns to work or is offered work for which the employee receives or would receive the same or greater salary, wages, or earnings than the employee received at the time of the injury, the employee shall be compensated based only upon the employee's functional impairment*

*resulting from the injury, and not in relation to the employee's earning capacity.*

The plain language of the statute provides no reasonable modifier regarding the offer of work. Without being able to read "suitability" into the offer of work, the results would allow for absurd offers to satisfy the statutory language in order to avoid fair compensation for injured workers.

"The question of whether an employer offered suitable work is ordinarily a fact issue." Neal v. Annett Holdings, Inc., 814 N.W.2d 512, 518 (Iowa 2012) (citing Schutjer v. Algona Manor Care Center, 780 N.W.2d 549, 557, 559 (Iowa 2010); McCormick v. N. Star Foods, Inc., 533 N.W.2d 196, 199 (Iowa 1995)). "The language of the statute requires that the work offered to an injured worker must be both 'suitable' and 'consistent with the employee's disability' before the employee's refusal to accept such work will disqualify him from receiving temporary partial, temporary total, and healing period benefits. *See Id.* Otherwise, the modifier 'suitable' would have no meaning and would be mere surplusage." Neal v. Annett Holdings, Inc., 814 N.W.2d at 520. In determining whether the work is suitable, Annett Holdings states that the workers' compensation agency may consider the definition of "suitable work" set forth in Iowa's unemployment statute:

*a. (1) In determining whether or not any work is suitable for an individual, the department shall consider the degree of risk involved to the individual's health, safety, and morals, the individual's physical*

> *fitness, **prior training**, length of unemployment, and prospects for securing local work in the individual's **customary occupation**, the distance of the available work from the individual's residence, and any other factor which the department finds bears a reasonable relation to the purposes of this paragraph. Iowa Code § 96.5(3)(a)(1); Annett Holdings, Inc., 814 N.W.2d at 522-23(emphasis added).*

The Defendants have the burden to prove that the jobs offered to Wooff were "suitable." Alonzo v. IBP, Inc., File No. 5009878 (10/31/06 App. Dec.). The employer must show by the preponderance of the evidence that the work was offered; that the work was suitable, that is, having a physical or **mental demand** level that does not exceed Wooff's capacities; and, that the refusal was an intentional act. Brodigan v. Nutri-ject Systems, Inc., File No. 5001106 (04/13/04 App. Dec.). Woods v. Siemens-Furnas Controls, File Nos. 1303082, 1273249 (07/22/03 App. Dec.).

Pursuant to Fix v. Iowa Workforce Development, 05/26/24 Appeal No. 24A-UI-06287-JT-T, Wooff would **NOT** have been found to have been earnestly looking for work and likely would **not have been awarded unemployment benefits** had he applied for the two jobs Defendants offered because he was not qualified for those jobs. Ms. Fix applied for one "Mid-senior level" job for which she did not meet the minimum qualifications. The ALJ concluded this application for a position for which she was not qualified was not an earnest search for suitable work.

Defendants did not present any evidence that Wooff was qualified to perform the jobs suggested. There is objective evidence to support his inability to do these jobs.

Wooff testified he was not capable or qualified in performing the estimator's job as it required computer work in which he has no experience. Both the estimator and superintendent jobs required strong computer skills, especially knowledge of spreadsheets (Cl. Ex. 7, pp. 295-297). When assessing the merits of the superintendent's job, Wooff's unrebutted testimony is that the job required supervising the two components of Menefee workers – the carpenters and the drywallers (Tr. p. 43). Wooff testified that Menefee is probably the largest drywall company in Cedar Rapids with multiple crews and over 100 employees (Tr. p. 23). As the superintendent, he would be responsible for determining direct and indirect staffing requirements, manage and oversee subcontractors, attend weekly and monthly job site progress meetings, manage job progression, oversee approximately 40 to 50 commercial job sites, and 90 to 120 tradesmen (Cl. Ex. 7, p. 295). He would also have to hire, train, and fire tradesmen (Cl. Ex. 7, p. 295). The position required detailed orientation, strong communication skills, and industry knowledge (Cl. Ex. 7, p. 295). Wooff had no experience in any supervisory roles.

Wooff obviously knew the work of a drywaller, but was unfamiliar with the work performed by carpenters and did not have the knowledge to supervise such (Tr. pp. 43-44). Review of the job descriptions requirements does not correlate with

Wooff's abilities (Cl. Ex. 7, pp. 295-297). These offers appear to be an attempt to satisfy legal issues rather than as bona fide job offers (Cl. Ex. 7, pp, 298-301).

Additionally, the offer of work was premature. Wooff was not physically capable of working at the time of the offer. Wooff was still treating the DENIED NECK INJURY (which was found to be compensable). He was not at maximum medical improvement for his work injuries at the time of the purported job offers made in August of 2021 (Defs. Ex. E, p. 68). He had his first cervical surgery 4 months later, on October 18, 2021 (Jt Med. Ex. 7, pp. 409, 429; Cl. Ex. 4, p. 156). He was unable to work from October 18, 2021, to March 2022, (5 months) and still has not been able to return to gainful employment. He attempted to work at Methwick in the spring of 2022 but was unable to physically perform the job due to his pain and need for ongoing care and he resigned in October of 2022 (Tr. pp. 40, 41, 65, 81; Cl. Ex. 4, p. 156). He then had a second cervical surgery on February 24, 2023, and had not been able to return to gainful employment at the time of the hearing (Jt. Med Ex. 7, pp. 530-531; Cl. Ex. 4, p. 156). Simply because the cervical injury was not accepted by Defendants does not mean they can escape the industrial disability analysis by offering him work while he was still treating for the denied portion of the claim.

The Commissioner held:

> *This case clearly involves a neck injury and a mental health injury. Neither of those conditions is accounted for, or is considered to be, a "scheduled member" injury pursuant to Iowa Code section 85.34(2)(a)-(u). Therefore, claimant has proven he sustained an "unscheduled" injury and that he should be compensated pursuant to Iowa Code section 85.34(2)(v).*
>
> *Iowa Code section 85.34(2)(v) provides, in pertinent part:*
>
> > *If an employee who is eligible for compensation under this paragraph returns to work or is offered work for which the employee receives or would receive the same or greater salary, wages, or earnings than the employee received at the time of the injury, the employee shall be compensated based only upon the employee's functional impairment resulting from the injury, and not in relation to the employee's earning capacity.*

(10/18/2024 App. Dec., p. 4).

These two paragraphs mean the neck injury is work-related and Wooff had reached MMI for it and therefore this is an industrial disability case. However, when the faced with the argument that AT THE TIME OF THE JOB OFFER, Wooff was not at MMI, the Commissioner held:

> *Once again, claimant reads something into the statute that does not exist in the plain language of the text. Nothing in Iowa Code section 85.34(2)(v) limits or specifies when an offer of work can or must be made by an employer. Nothing in the plain language of the statute requires the job offer only be made after maximum medical improvement or that "another offer of work" be made after maximum medical improvement.*

(11/12/24 Ruling on Cl's Motion for Rehearing of App. Dec., p. 2).

However, 85.34(2)(v) absolutely does require that the injured worker be at MMI when determining whether they are entitled to only the functional rating because they were offered a job or industrial disability because they were not. In this case, the job offer was made prior to Wooff being at MMI for his neck. While he was at MMI for his shoulder when the offer was made, he was not at MMI for the entire work-related injury. The Commissioner's conclusion above ignores the fact that Wooff would not have been able to work from October 18, 2021, to March 2022, and again after his second surgery in February 2023. The neck condition was DENIED by the Defendants and Wooff would not have received workers' compensation benefits during that period and likely would have been terminated due to his inability to be at work as their SUPERINTENDENT. If the Commissioner can speculate that Wooff was going to be trained to be able to do the superintendent job he can also speculate that he would have lost that job due to the amount of work he would have missed for a condition the employer did not agree was work-related.

It was irrational, illogical, or wholly unjustifiable and error of law to conclude that an offer of work for which Wooff was unqualified and mentally unable to perform was sufficient to avoid the industrial disability analysis. Furthermore, the offer was made when Wooff had not reached MMI for his work-related neck injury and was premature. If the statute does not associate the offer of return to suitable

employment with maximum medical improvement, the result is absurd. The statute clearly reads the offer of employment is associated with compensation under § 85.34(2)(v), wherein compensation is associated with a permanent injury. How can an offer of work be deemed appropriate when the Wooff is scheduled for surgery for his cervical spine, shortly after the offer, and the cervical spine condition was being denied, but found compensable by the final agency decision? At the time of the hearing, Wooff had reached MMI for his neck but no offer of work within his restrictions and education/experience had been made. Dr. Woods stated that Wooff is unable to work. The Commissioner circumvented this issue by finding that Wooff was limited to his functional rating only based upon the job offer (and then also, as discussed further below, disregarded Dr. Wood's impairment rating when calculating Wooff's functional impairment percentage). Under these facts, Wooff should have been awarded industrial disability rather than limited to the functional impairment.

### B. SUBSTANTIAL EVIDENCE SUPPORTS THAT WOOFF'S MENTAL HEALTH INJURY SHOULD BE CONSIDERED IN THE INDUSTRIAL DISABILITY ANALYSIS

With regards to Wooff's mental health injury, Commissioner stated:

> *Having found the deputy commissioner accurately concluded claimant sustained right shoulder, neck,* ***and mental health injuries*** *as a result of his work activities, I next consider the permanent total disability*

*claim asserted by claimant* (10/18/24 App. Dec., p. 2) (emphasis added).

The Commissioner then summarily concluded:

*With respect to the mental health injury, David {sic} Woods, M.D., J.D., offered an analysis about whether claimant would be able to return to work. However, Dr. Woods did not assign any permanent restrictions for the mental health injury. Claimant also returned to work for a different employer and did not identify mental health as a reason he had to quit that subsequent position.* (10/18/24 App. Dec., pp. 3-4).

Finally, the Commissioner held:

*This case clearly involves a neck injury and a mental health injury. Neither of those conditions is accounted for, or is considered to be, a "scheduled member" injury pursuant to Iowa Code section 85.34(2)(a)-(u). Therefore, claimant has proven he sustained an "unscheduled" injury and that he should be compensated pursuant to Iowa Code section 85.34(2)(v)*

*...*

*With respect to claimant's mental health issues, I affirm the deputy commissioner's rejection of the permanent impairment rating offered by Dr. Segal. Ultimately, Dr. Woods imposes no permanent work restrictions. I find claimant failed to prove specific permanent functional impairment under the AMA Guides to the Evaluation of Permanent Impairment, Fifth Edition, related to the mental health injury* (10/18/24 App. Dec., pp. 4-5).

The Commissioner either missed or ignored, without explanation, the following conclusion by Dr. Woods regarding Wooff's permanent impairment from his mental health injury:

*In terms of a "functional impairment rating", I would assign Mr. Wooff a 40% functional loss as a result of the work-related injury. This is due to his Class 3 "Moderate Impairment" and is modeled off the decision of*

> *"Clark v. Arconic, Inc." where Mr. Joseph Cortese, the Worker's Compensation Commissioner, found for a 30% functional loss for an individual deemed to have mild impairment. In addition, Table 11-4 from the AMA Guides was used as a model to help arrive at this rating. This is because (A) the AMA Guides do not provide rating percentages for mental health and (B) in Table 11-4 (like in Table 14-1) the Class 1 impairment is "no impairment". This MD therefore thought it prudent to use Table 11-4 as a model to provide at least some guidance for providing a numeric rating.* (Cl. Ex. 2, p. 140).

Regarding Wooff's employability and restrictions, Dr. Woods opined:

> *With all that said, this MD is highly doubtful that Mr. Wooff will be able to work a physical labor job ever again. Given his age and education level, it also seems improbable that he will be able to retrain for a non-physical, primarily cerebral line of work either. For the sake of his quality of life and peace of mind, we must proceed forward under the assumption he is not able to work at all at this point in time. This MD is not comfortable, however, assigning permanent restrictions/limitations and will defer to Dr. Alden, the VA mental health staff, and my physical medicine colleagues to make such decisions* (Cl. Ex. 2, p. 141).

It's unclear how the statement "…this MD is highly doubtful that Mr. Wooff will be able to work a physical labor job ever again does not equate to "restrictions."

Dr. Woods stated:

> 7. *What effects, if any, would your diagnoses have on Mr. Wooff's employability (mood, concentration, energy, ability to work with others, memory, ability to attend work on a regular basis)?*
>
> *Mr. Wooff's Adjustment Disorder presents him with significant hindrances in all the domains mentioned in this question. Especially in light of his chronic pain, weakness, and mobility issues, the client's employability is greatly reduced by his labile moods, reduced concentration and energy, and his overall rapid exhaustion from dealing*

*with the mental and physical anguish would make it very difficult for him to attend work with any reliability. (Cl. Ex. 2, p. 139*).

Dr. Woods also stated, "Given his age and education level, it also seems improbable that he will be able to retrain for a non-physical, primarily cerebral line of work either." (Cl. Ex. 2, p. 141). The only evidence offered by Defendants regarding Wooff's mental health was Dr. Carpenter's report after never meeting with Wooff.

The Commissioner completely disregarded Dr. Woods's rating for Woolf's mental health – apparently because there were no specific restrictions. What restrictions did the Commissioner expect to see pertaining to depression – a limitation on how many hours Wooff can concentrate, how many numbers Wooff can crunch within an hour, and to allow him to miss work on an as-needed basis?

When pushed by Wooff in an Application for Rehearing for further explanation as to why Dr. Woods's 40% impairment rating was not awarded the Commissioner simply stated:

*Claimant's final challenge in his motion for rehearing is for "a rehearing on the impact of Dr. Woods's 40% permanent impairment rating." No further explanation of this challenge is made in the motion for rehearing. No explanation as to how the agency erred in the appeal decision is made.*

*For purposes of this rehearing, I note Dr. Woods' opinions were acknowledged in the appeal decision. While every medical opinion was not quoted at length in the appeal decision, a de nova review of the*

*record was made on appeal. Therefore, Dr. Woods's opinions were considered on intra-agency appeal.*

(11/12/2024 Ruling on Cl's Motion for Rehearing of App. Dec., p. 4).

Due to the non-existence of a mechanism for rating mental impairment in the AMA Guides, the Commissioner took it upon himself to create an impairment rating for a mental health injury in which the record only contained one rating of 100%. In Clark v. Arconic, File No 5061553.01 (11/15/21 Arb. Dec.), the Deputy awarded Clark 100% impairment based upon a 100% rating by Dr. Thomas, when Dr. Brooke and Dr. Jennisch offered no percentage of impairment. In doing so, the Deputy stated, "However, given that Iowa Code Section 85.34(2)(x) demands that the undersigned use no agency expertise in determining loss or percentage of impairment when determining functional disability, Dr. Thomas' disability rating is adopted herein." On appeal, the Commissioner CREATED AN IMPAIRMENT RATING SYSTEM FOR MENTAL HEALTH INJURIES and stated:

*The deputy commissioner found that because Iowa Code section 85.34(2)(x) demands she use no agency expertise in determining loss or percentage of impairment when determining functional disability, Dr. Thomas's disability rating of a 100 percent impairment must be adopted because C. Scott Jennisch, M.D., did not offer a permanent impairment rating. I respectfully disagree with the deputy commissioner's findings with the following substituted findings and analysis.*

*…*

*The issue of how to determine functional loss for a work-related mental health injury raises an issue of statutory interpretation. In Iowa, the*

*primary purpose of the workers' compensation statutes is to benefit the injured worker. Denison Mun. Util. v. Iowa Workers' Comp. Comm'n, 857 N.W.2d 230, 234-235 (Iowa 2014) The courts liberally construe the workers' compensation statutes in favor of the injured worker. Id.*

*...*

*The statute in this case requires the use of the AMA Guides in determining functional loss. Dr. Jennisch properly noted the AMA Guides direct the examiner to not assign a permanent impairment rating, but rather to assess and record the extent of the individual's functioning.*

*...*

*Dr. Thomas and Dr. Jennisch assessed and recorded their observations of claimant's functioning and assigned a class of impairment for each category. While I disagree, as the deputy did, with Dr. Jennisch's finding of secondary gain, I find Dr. Jennisch's assessment of claimant's functioning most consistent with the record evidence.*

*...*

*Claimant has continued to work full-time for defendant-employer since the work injury…I find the evidence supports Dr. Jennisch's finding that claimant has sustained a mild level of impairment in activities of daily living.*

*Using the criteria set forth in the AMA Guides, the evidence is consistent with Dr. Jennisch's findings that claimant has mild limitations in functioning. Id. Therefore, considering the record as a whole, I find claimant has sustained 30 percent functional loss as a result of the work injury.*

Clark v. Arconic, File No 5061553.01 (11/15/21 Arb. Dec.).

Interestingly, unlike Wooff, Clark did not have any formal restrictions for her mental health and was still working full-time, making more money. Yet the Commissioner found Clark to have 30% impairment (reducing it from 100%) due to her mental health injury, but in this case disregarded Dr. Woods 40% rating for

Wooff. Dr. Woods even referred to the Clark decision, and utilized the Commissioner's model, in rendering his impairment rating wherein he stated:

> *In terms of a "functional impairment rating", I would assign Mr. Wooff a 40% functional loss as a result of the work-related injury. This is due to his Class 3 "Moderate Impairment" and is modeled off the decision of "Clark v. Arconic, Inc." where Mr. Joseph Cortese, the* Worker's *Compensation Commissioner, found for a 30% functional loss for an individual deemed to have mild impairment. In addition, Table 11-4 from the AMA Guides was used as a model to help arrive at this rating. This is because (A) the AMA Guides do not provide rating percentages for mental health and (B) in Table 11-4 (like in Table 14-1) the Class 1 impairment is "no impairment". This MD therefore thought it prudent to use Table 11-4 as a model to provide at least some guidance for providing a numeric rating.* (Cl. Ex 2, p. 140).

Substantial evidence supports that Wooff's mental health injury resulted in significant permanent impairment, as determined by Dr. Woods, pursuant to the Commissioner's prior decisions, and should be considered in the industrial disability analysis.

### C. THE COMMISSIONER ERRED AS A MATTER OF LAW WHEN IT IMPROPERLY USED AGENCY EXPERTISE IN DETERMINING WOOFF'S PERMANENT IMPAIRMENT

If it is determined that Wooff is limited to recovering under the functional impairment analysis, the Commissioner improperly used agency expertise in determining Wooff's permanent impairment.

Iowa Code § 85.34(2)(x) states:

> *In all cases of permanent partial disability described in paragraphs "a" through "u", or paragraph "v" when determining functional disability and not loss of earning capacity, the extent of loss or percentage of permanent impairment shall be determined solely by utilizing the guides to the evaluation of permanent impairment, published by the American medical association, as adopted by the workers' compensation commissioner by rule pursuant to chapter 17A. Lay testimony or agency expertise* ***shall not*** *be utilized in determining loss or percentage of permanent impairment pursuant to paragraphs "a" through "u", or paragraph "v" when determining functional disability and not loss of earning capacity. (emphasis added).* [1]

The words "shall not" impose a mandatory duty not to do something – a mandatory duty NOT TO USE AGENCY EXPERTISE IN DETERMINING PERMANENT IMPAIRMENT. The legislature made this clear in drafting the Iowa Code when it said the use of "shall" imposes a duty;" while "may" confers a power. Iowa Code § 4.1(30)(a)(c)(1997); State v. Klawonn, 609 N.W.2d 515, 521–22 (Iowa 2000). Additionally, the Supreme Court has interpreted the term "shall" in a statute to create a mandatory duty, not discretion. *See* State v. Moyer, 382 N.W.2d 133, 134–35 (Iowa 1986); *see also* State v. Luckett, 387 N.W.2d 298, 301 (Iowa 1986) (use of "shall" creates mandatory action unless context clearly indicates otherwise). In Hansen v. Henderson, 244 Iowa 650, 665, 56 N.W.2d 59, the Supreme Court said:

> *Sometimes courts are justified in interpreting the word 'shall' as 'may,' but, when used in a statute directing that a public body do certain acts,*

[1] Paragraphs "a" through "u" and "v" are called "scheduled injuries" for which there is a specified number of weeks allocated to body parts and for which loss of earning capacity is not awarded in most circumstances. The award is limited to the percentage of weeks assigned to that body part. That percentage is called an "impairment rating."

> *it is manifest that the word is to be construed as mandatory and not permissive. The uniform rule seems to be that the word 'shall,' when addressed to public officials, is mandatory and excludes the idea of discretion. Id. at 67 (1952).*

The Commissioner went to great lengths to dissect the impairment rating reports, utilizing agency expertise to pick and choose parts of the ratings to trust and other parts to disregard. Regarding Wooff's treating physician, Dr. Nepola, the Appeal Decision states,

> *"I find James Nepola, M.D.'s permanent work restrictions for the shoulder to be convincing and appropriate. Dr. Nepola is highly qualified orthopaedic shoulder surgeon and provided treatment for claimant. Dr. Nepola's restrictions pertaining to repetitive reaching away from the body or above shoulder height are not sufficient to preclude claimant from alternate employment* (10/18/24 App. Dec., p. 3).

However, the Appeal Decision does not acknowledge the following opinion by treating physician, Dr. Nepola:

> *To the nearest degree of medical certainty Michael D Wooff has a permanent partial impairment rating of 16% of the upper extremity which is equivalent to 10% of the whole person according to the Guides to Evaluation of Permanent Partial Impairment of the AMA, 5th Edition. This rating is the result of loss of active forward flexion (1% upper extremity) and extension (1% upper extremity) per figure 16-60 on page 476, loss of active external rotation (1% upper extremity) and internal rotation (2% upper extremity) per table 16-46 on page 479, loss of abduction (0% upper extremity) and adduction (1% upper extremity) per Figure 16-43 on Page 477 and distal clavicle resection arthroplasty (10% upper extremity ) per Table 16-27 on page 506 the Guides. The figure for impairment due to loss of range of motion (6% upper extremity) were combined with the loss due to the distal clavicle resection arthroplasty (10% upper extremity) per the Combined Values Chart on*

*Page 605-605 the Guides. The upper extremity impairment (16%) was converted to impairment to the whole person per Table 16-3 on page 439 of the Guides. This is intended to be an assessment of the function loss of Michael D. Wooff's shoulder on 8/10/21, and not intended to be combined with any previously assigned impairment rating* (Jt. Ex. 7, p 387).

On appeal, the Commissioner concluded:

*The deputy commissioner accepted and adopted the restrictions from David Segal, M.D., pertaining to claimant's neck injury. I accept those restrictions as well. The restrictions for claimant's neck are more limiting than the shoulder restrictions.*

*...*

*With respect to functional disability, I find the shoulder impairment offered by Joseph Chen, M.D., to be most convincing. Dr. Nepola is claimant's treating shoulder surgeon, he had multiple opportunities to evaluate claimant's ranges of motion, understands claimant's shoulder having performed surgery, and identified loss of range of motion equivalent to six percent of the right upper extremity. (Joint Ex. 7, p 387) Dr. Chen concurred with these range of motion measurements and impairment and I find claimant proved six percent functional loss of the right upper extremity as a result of a loss of range of motion in the right shoulder. Dr. Chen also offered an additional three percent permanent impairment of the right upper extremity for the distal clavicle excision performed in claimant's right shoulder. I accept Dr. Chen's three percent permanent impairment rating for the distal clavicle excision as well, In total, I find claimant proved a nine percent permanent impairment of his right upper extremity. (Defendants' Ex. C, p. 37) This converts to a five percent permanent functional impairment of the whole person pursuant to Table 16-3 located on page 439 of the AMA Guides to the Evaluation of Permanent Impairment, Fifth Edition.*

*Dr. Segal is a neurosurgeon and not a shoulder specialist. Moreover, his impairment rating for the shoulder is inflated and is inconsistent with prior agency precedent pertaining to impairment related to a distal*

> *clavicle excision*[2]*. However, the deputy commissioner relied upon Dr. Segal's opinions with respect to the neck injury. I find Dr. Segal's permanent impairment rating for claimant's neck injury is reasonable and convincing. Therefore, I find claimant proved 35 percent whole person permanent functional impairment related to the neck injury. (Claimant's Ex. 1, p. 67)*
>
> *With respect to claimant's mental health issues, I affirm the deputy commissioner's rejection of the permanent impairment rating offered by Dr. Segal. Ultimately, Dr. Woods imposes no permanent work restrictions. I find claimant failed to prove specific permanent functional impairment under the AMA Guides to the Evaluation of Permanent Impairment, Fifth Edition, related to the mental health injury.*

(10/18/24 App Dec., pp. 3, 5).

The Commissioner disregards Dr. Segal's impairment for the shoulder because "he's not a shoulder specialist." Dr. Chen is also NOT a shoulder specialist. However, Dr. Nepola, who was the authorized treating physician, IS A SHOULDER SPECIALIST. Dr. Nepola found Wooff to have impairment of 16% of the upper extremity which is equivalent to 10% of the whole person regarding his shoulder.

Second, regarding the statement that Dr. Segal's rating is "inconsistent with agency precedent" - "agency precedent" does not decide impairment – doctors decide impairment utilizing the AMA Guides. If agency precedent determined

[2] "Agency precedent pertaining to impairment related to a distal clavicle excision" was formulated by the Commissioner in Jay v. Archer Skid Loader Service, File No 19003586.01 (08/23/22 App. Dec.) wherein the Commissioner concluded (without medical evidence in the record to support it) that a distal clavicle excision is limited to a 3% impairment. As acknowledged by the Commissioner in his Ruling on Claimant's Motion for Rehearing, this improper use of agency expertise is the issue of numerous pending appeals.

ratings, we would not need the Deputies or Commissioners for most cases. Finally, the Commissioner completely disregarded Dr. Woods's impairment rating for Wooff's mental injury.

Pursuant to Iowa Administrative Procedure under Rules of Evidence, Section 17A.14(5), the Agency may utilize its experience, technical competence, and specialized knowledge when **evaluating the evidence** (emphasis added). The agency's experience is to be "utilized in the evaluation of the evidence." IBP, Inc. v. Al-Gharib, 604 N.W.2d 621 (Iowa 2000). They are limited to evaluating the EVIDENCE THAT IS IN THE RECORD. When explaining who performs impairment evaluations, the AMA Guides to the Evaluation of Permanent Impairment, Fifth Edition ("Guides"), state: "Impairment evaluations are performed by a licensed physician" (Guides, p. 18). Impairment evaluations and determining **which sections of the Guides are applicable is not uniquely within the subject matter expertise of the Agency nor has the Agency been vested with interpretative authority**. Renda v. Iowa Civil Rights Comm'n, 784 N.W.2d 8, 14 (Iowa 2010); Neal v. Annett Holdings, Inc., 814 N.W.2d 512 (Iowa 2012) (emphasis added).

The Agency recently held, consistent with Iowa Code § 85.34(2)(x), that impairment is to be determined by using the **Guides** without reliance on lay

testimony or agency expertise. Ahrens v. Earwood Family Properties, LLC, File No 5066611 (08/31/21 App. Dec.). "The overwhelming majority of injuries are given impairment ratings using formulas and tables in the Guides that are nuanced and complicated and require reliance on subjective complaints, in-office measurements, or other factors that are **only appropriately considered by a physician**." Id. (emphasis added).

Courts are reluctant to allow the Commissioner to totally reject expert testimony when it is the only medical evidence presented. Poula v. Siouxland Wall & Ceiling, Inc., 516 N.W.2d 910, 911-912 (Iowa Ct. App. 1994). And while it is true that generally the Commissioner may accept or reject expert testimony entirely or in part, the Commissioner may not arbitrarily or totally reject the testimony but must weigh the evidence and assess the credibility of the witness. Lithcote Co. v. Ballenger, 471 N.W.2d 64, 66 (Iowa App. 1991); Catalfo v. Firestone Tire & Rubber Co., 213 N.W.2d 506, 509 (Iowa 1973). Leffler v. Wilson & Co., 320 N.W.2d 634, 635 (Iowa App. 1982); Langford v. Kellar Excavating & Grading, Inc., 191 N.W.2d 667, 668 (Iowa 1971). The stated reasons for the rejection of such opinions must be valid. Bensley v. Dee Zee Mfg., 807 N.W.2d 158 (Iowa Ct. App. 2011).

The Commissioner erred as a matter of law by using Agency expertise contrary to Iowa Code section 85.34(2)(x). *See* Severin v. Second Injury Fund of

Iowa, (Polk Co. Case No. CVCV 063424, 02/14/23 Ruling on Petition for Judicial Review). In Severin, the Agency rejected Dr. Hines's impairment rating of the claimant. Id. at 5. The District Court held that the rejection of Dr. Hines's unopposed impairment rating was based upon the Agency's "agency expertise" and, thus, an error of law. Id. at 5-6.

> *Here, the Commission abused its discretion by employing "agency expertise" in its rejection of Dr. Hines' impairment rating of Severin. The Commission held that Table 17-31 of the Guides, upon which Dr. Hines relied, does not apply to claimant's alleged injury. (See Ruling on Motion for Rehearing at 2; Arbitration Decision at 6; CR at 6, 57). However, there was no expert opinion to that effect. Instead, it was the Deputy Commissioner and Commissioner's own opinion that Table 17-31 regarding arthritis impairments based upon roentgenographically determined cartilage intervals does not apply in this case.*
>
> *Certainly, the Commission is not required to accept an expert opinion simply because there is no contrary expert opinion. "The commissioner, as the fact finder, determines the weight to be given to any expert testimony. Such weight depends on the accuracy of the facts relied upon by the expert and other surrounding circumstances. The commissioner may accept or reject the expert opinion in whole or in part."* Schutjer v. Algona Manor Care Ctr.*, 780 N.W.2d 549, 560 (Iowa 2010);* see also Sanchez v. Blue Bird Midwest*, 554 N.W.2d 283, 285 (Iowa Ct. App. 1996) ("Expert opinion testimony, even if uncontroverted, may be accepted or rejected in whole or in part by the trier of fact."). Therefore, the Commission may reject an expert opinion because of a factual determination or credibility determination that is foundational to the expert opinion, even if the expert opinion is uncontroverted.*
>
> *Here, however, the Commission did not act based solely on a foundational factual or credibility determination. Instead, the Commission relied on the agency's own expertise in applying the AMA*

> *Guides to disagree with Dr. Hines' application of the Guides. This is an action that is specifically prohibited by Iowa Code section 85.34(2)(x).* Id. at 5 (emphasis added)(internal citations and quotations omitted).

Wooff submits that when the agency modifies an impairment rating rendered by a physician, this constitutes the use of agency expertise. What qualifies a Deputy or the Commissioner to do this if not "agency expertise?"

Wooff submits the application of the AMA Guides is subject to interpretation ONLY by a medical doctor and IS NOT to be decided using agency expertise. Prior to the 2017 legislative amendment prohibiting use of agency expertise and lay testimony, it was common to have evidence of two competing impairment ratings and the agency could award something in between. This is exactly what the legislative change attempted to eliminate and why the legislature disallowed the use of "agency expertise" and "lay testimony" in determining impairment ratings. The goal was to remove any "subjectivity" in determining impairment and to make determining such completely dependent on the impairment ratings rendered by the physicians. The Agency still has the job of "evaluating the evidence" and weighing the expert opinions but now the Agency must simply choose one expert opinion on impairment from the evidence in the record without reference to the AMA Guides.

A similar issue was addressed by the Iowa Court of Appeals in Harrell v. Denver Findley & Sons, Inc., 986 N.W.2d 872 (Iowa Ct. App. 2022) when the

Second Injury Fund wanted credit for a claimant's prior total knee replacement. The court determined that evidence of the claimant's prior knee replacement was NOT IN THE RECORD and therefore, could not be utilized in determining impairment and credit. The Court stated:

> *In 2017, the legislature enacted H.F. 518, an act relating to workers' compensation. 2017 Iowa Acts ch. 23, § 9 (codified at Iowa Code § 85.34(2)(x)). As codified, it includes this provision:*
>
> *In all cases of permanent partial disability described in paragraphs "a" through "u", or paragraph "v" when determining functional disability and not loss of earning capacity, the extent of loss or percentage of permanent impairment shall be determined solely by utilizing the guides to the evaluation of permanent impairment, published by the American medical association, as adopted by the workers' compensation commissioner by rule pursuant to chapter 17A. Lay testimony or agency expertise shall not be utilized in determining loss or percentage of permanent impairment pursuant to paragraphs "a" through "u", or paragraph "v" when determining function disability and not loss of earning capacity. Iowa Code § 85.34(2)(x) (emphasis added). The parties agree that—for present purposes—the "guides" means the Fifth Edition of the American Medical Association's Guides to the Evaluation of Permanent Injury (Guides).*
>
> *With this background, we consider the commissioner's decision on the credit issue. First, the commissioner found "it is undisputed" that Harrell "underwent a total knee replacement prior to" his 2018 foot injury. The commissioner then consulted Table 17-33 in the Guides, which is entitled "Impairment Estimates for Certain Lower Extremity Impairments." Among other things, Table 17-33 includes impairment estimates for various knee procedures. For a "total knee replacement," the Table suggests impairment percentages of thirty-seven for a "good result," fifty for a "fair result," and seventy-five for a "poor result." So, as the commissioner reasoned, the Table requires a minimum "impairment rating of 37[%] for a total knee replacement."*

*Accordingly, the commissioner concluded, "the Fund is entitled to" a credit "for the 37[%] permanent disability of [Harrell's] left total knee replacement."*

*Harrell raises two objections to this approach. First, he suggests that only a physician can fulfill the statutory requirement of determining "the extent of loss or percentage of permanent impairment . . . solely by utilizing the" Guides. As Harrell notes, the Guides themselves expressly anticipate that physicians will determine impairments. See Am. Med. Ass'n, Guides to the Evaluation of Permanent Impairment 3 (Linda Cocchiarella & Gunnar B.J. Andersson eds., 5th ed. 2001) (noting that, under the Guides, the physician's role is to determine impairment).*

*Harrell also suggests that when a non-physician commissioner or deputy commissioner uses the Guides to determine impairment, they necessarily rely on "agency expertise"—a source forbidden by section 85.34(2)(x).*

*For purposes of this appeal, we need not decide whether non-physicians may properly use the Guides to determine impairment. We leave that question for another case. We focus instead on Harrell's argument that the commissioner's thirty-seven percent impairment determination was inconsistent with section 85.34(2)(x)'s prohibition on the use of "lay testimony" to determine "loss or percentage of permanent impairment." On this point, we believe Harrell is correct. The commissioner's impairment determination was built on the assumption that Harrell had undergone a total knee replacement surgery. But no physician or other expert testified about a total knee replacement surgery. And no records from a total knee replacement surgery were entered as exhibits. The only evidence of a total knee replacement surgery is Harrell's own testimony—the very "lay testimony" forbidden by section 85.34(2)(x). So it appears the commissioner's determination violated the statute.*

*...*

*The Fund also suggests that, by precluding reliance on lay testimony, section 85.34(2)(x) "make[s] it both more costly and more challenging for a claimant to establish a permanent injury and entitlement to permanency benefits." Policy arguments like those are*

> *better directed to the people's elected representatives. See Iowa Const. art. III, § 1 (prohibiting members of the judicial branch from exercising powers of the legislative branch or the executive branch). "The court's job is to carry out the legislative project, not to change it in conformity with the judge's view of sound policy."* Frank H. Easterbrook, *Foreword* to Antonin Scalia & Bryan Garner, *Reading Law: The Interpretation of Legal Texts*, at xxi (2012).

Harrell v. Denver Findley & Sons, Inc., 986 N.W.2d 872 (Iowa Ct. App. 2022).

The Court in Harrell strictly construed the words of the statute and prohibited consideration of the knee replacement because no physician or expert testified about it and so to consider it the Agency had to use "lay testimony" – which is prohibited.

The Commissioner erred as a matter of law when he used agency expertise to determine Wooff's permanent impairment. The Commissioner ignored treating orthopedical shoulder specialist Dr. Nepola's 16% shoulder (10% body as a whole) rating because he did not agree with the distal clavicle excision rating and COMPLETELY IGNORED Dr. Woods's 40% body as a whole rating for Wooff's mental health. IF it is determined that Wooff's recovery is limited to functional impairment, he should be awarded either Dr. Segals 13% BAW OR Dr. Nepola's 10% BAW for the shoulder injury, Dr. Segal's 35% BAW for the cervical spine, and Dr. Woods's 40% BAW for the mental health injury.

## CONCLUSION

Wherefore, Wooff requests the Court find that the offer of work made by Defendants was not suitable and remand the matter back to the Agency with instructions to use the industrial disability analysis and for such analysis to include consideration of Dr. Nepola and Dr. Segal's distal clavicle excision ratings and Dr. Woods's permanent impairment rating for Wooff's mental health injury. In the alternative, if it is found that the basis of Wooff's compensation is his functional impairment rating, then the matter should be remanded back with instructions to utilize the appropriate impairment rating for the distal clavicle excision, Dr. Segal's 35% BAW for the cervical spine, and the mental health rating by Dr. Woods.

## REQUEST FOR ORAL ARGUMENT

Petitioner-Appellant, Michael Wooff, hereby requests this matter be set for Oral Argument to further discuss the issues on appeal.

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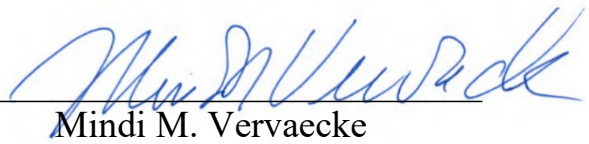
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Mindi M. Vervaecke

## CERTIFICATE OF FILING

I, Denise M. Wall, an employee of Wertz Law Firm, attorneys for the Appellee, certify the attached Brief was filed with the clerk of the Iowa Supreme Court via electronic filing on this 29th day of October 2025.



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## CERTIFICATE OF SERVICE

I, Denise M. Wall, an employee of Wertz Law Firm, attorneys for the Appellees, certify the attached Brief filed with the Clerk of the Iowa Supreme Court was forwarded to all counsel via the electronic filing system and email on this 29th day of October, 2025, and by U.S. Mail for any party not registered to receive notice of filings via the electronic filing process.

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